

smart Health Report

An Insightful Health Analytics Report
for Easier Understanding

Prepared For

Mr RAJORSHI SEAL

M 54



Name
Mr RAJORSHI SEAL

Gender
M

Patient ID
16370278

Age
54

Your Health at a Glance – A Personalized Journey

Report Sections

- 1 **Body Summary**
A visual snapshot of your overall health, simple and easy to understand
- 2 **Quick Health Highlights**
Your health scores and a single view of all abnormal results for quick attention
- 3 **Lab Report Overview**
Understand at a glance which tests are normal and which are abnormal
- 4 **Personalized Health Advisory**
Actionable insights and expert guidance tailored just for you
- 5 **Doctor's Reference Report**
Complete lab results with interpretations to share with your healthcare provider

How to Read This Report

This comprehensive health report provides detailed insights into your test results. Each section offers different perspectives on your health status, from visual summaries to detailed analysis and personalized recommendations.



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● All In Range ● Borderline ● Out Of Range

Health Summary



CARDIAC PROFILE

Test Name	Result
Triglycerides	198
Non HDL Cholesterol	141.3
LDL Cholesterol	101.7
+ 1 tests Please Watchout	



KIDNEY PROFILE

Everything looks good



ELECTROLYTES

Everything looks good



VITAMIN PROFILE

Test Name	Result
Vitamin D 25 - Hydroxy	17.7
Please Watchout	



THYROID PROFILE

Everything looks good



DIABETES MONITORING

Test Name	Result
HbA1c (Glycosylated Haemoglobin)	5.9
Please Watchout	



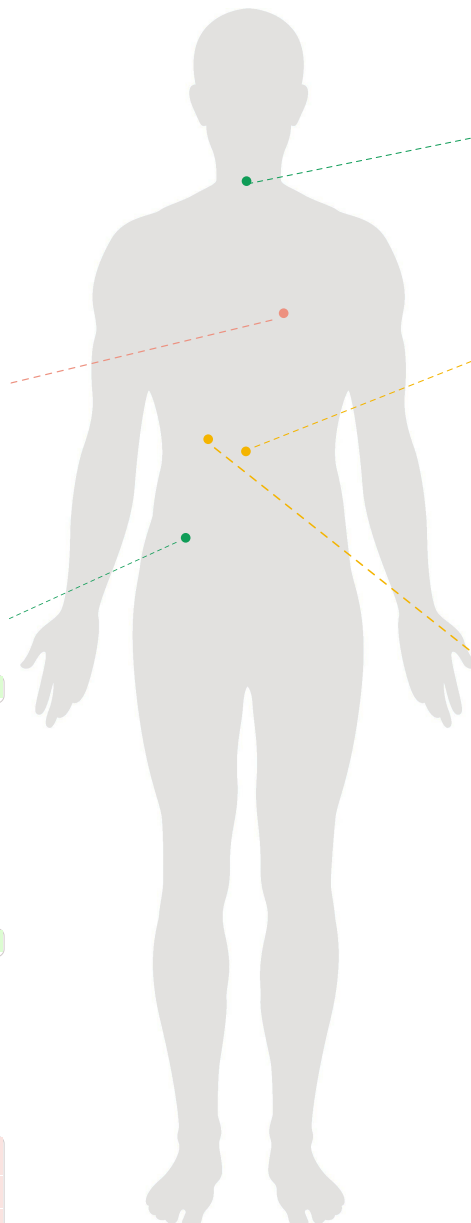
LIVER PROFILE

Test Name	Result
GGT	11.2
Please Watchout	



ANEMIA STUDIES

Test Name	Result
RDW-SD	51.8
Please Watchout	



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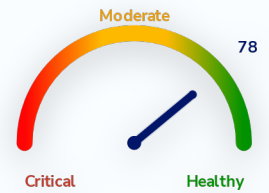
Quick Health Summary

Personal Insights - Health Score

78

Overall, most parameters are within normal ranges, indicating good general health. The Cardiac Health and Iron profiles may affect energy levels, while the Vitamins and Minerals profile suggests room for nutritional improvement. Consider incorporating a variety of fruits, vegetables, and balanced meals into your diet, along with regular physical activity such as walking or yoga. Routine check-ups can help maintain your well-being, and please consult a healthcare professional if you notice any new or concerning symptoms. Remember, small lifestyle changes can lead to meaningful health benefits.

Note - Higher scores tentatively indicate better health status



Summary of Key Health Indicators

Total Parameters Tested	Borderline Results	Out Of Range Results
74	5	5

Health Status by Body System

Profile	Total	Borderline	Out of Range	Key Results
Cardiac Profile	9	2	2	- Triglycerides (198) - VLDL (39.6) - Non - HDL Cholesterol (141.3) - LDL Cholesterol (101.7)
Blood Disorder	16	0	1	Abs. Basophil Count (0.01)
Iron	4	0	1	UIBC (309)
Vitamin Profile	2	0	1	Vitamin D (25-Hydroxy) (17.7)
Infectious Diseases	2	0	0	All In Range
Anemia Studies	8	1	0	RDW-SD (51.8)
Inflammation	1	0	0	All In Range
Diabetes Monitoring	3	1	0	HbA1c (Glycosylated Haemoglobin) (5.9)
Liver Profile	12	1	0	GGT (11.2)
Kidney Profile	8	0	0	All In Range
Electrolytes	4	0	0	All In Range



Profile	Total	Borderline	Out of Range	Key Results
Thyroid Profile	4	0	0	All In Range



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Report Summary

● In Range● Borderline● Out Of Range

No color - Reference range not available

INFECTIOUS DISEASES

Test Name	Result unit	Range
HEPATITIS B SURFACE ANTIGEN (HBsAg)	NON REACTIVE S/co	
● PCT	0.2 %	0.17 - 0.32

ANEMIA STUDIES

Test Name	Result unit	Range
● Hemoglobin	13.9 g/dL	13 - 17
● PCV	41.2 %	40 - 50
● MCV	94.6 fL	83 - 101
● MCH	32 pg	27 - 32
● MCHC	33.8 g/dL	31.5 - 34.5
● RDW (CV)	13.8 %	11.6 - 14
● RDW-SD	51.8 fL	35.1 - 43.9
Mentzer Index	21.5 %	

BLOOD DISORDER

Test Name	Result unit	Range
● TLC	6.8 $10^3/\mu\text{L}$	4 - 10
● Neutrophils	64 %	40 - 80
● Lymphocytes	28.8 %	20 - 40
● Monocytes	4.9 %	2 - 10
● Eosinophils	2.1 %	1 - 6
● Basophils	0.2 %	< 2
● Neutrophils.	4.35 $10^3/\mu\text{L}$	2 - 7
● Lymphocytes.	1.96 $10^3/\mu\text{L}$	1 - 3
● Monocytes.	0.33 $10^3/\mu\text{L}$	0.2 - 1
● Eosinophils.	0.14 $10^3/\mu\text{L}$	0.02 - 0.5
● Basophils.	0.01 $10^3/\mu\text{L}$	0.02 - 0.5
● Platelet Count	196 $10^3/\mu\text{L}$	150 - 410
● Mean Platelet Volume (MPV)	11 fL	9.3 - 12.1
● PDW	19.8 fL	8.3 - 25
● P-LCR	43.2 %	18 - 50
● P-LCC	63 $10^9/\text{L}$	44 - 140

INFLAMMATION

Test Name	Result unit	Range
● ESR - Erythrocyte Sedimentation Rate	08 mm/hr	< 12



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● In Range

● Borderline

● Out Of Range

No color - Reference range not available

DIABETES MONITORING

Test Name	Result unit	Range
● Glycosylated Hemoglobin (HbA1c)	5.9 %	< 5.6
Estimated Average Glucose	122.63 mg/dL	
● Glucose Fasting	96.5 mg/dL	70 - 100

LIVER PROFILE

Test Name	Result unit	Range
● Bilirubin Total	0.331 mg/dL	0.2 - 1.2
● Bilirubin Direct	0.138 mg/dL	< 0.5
● Bilirubin Indirect	0.19 mg/dL	0.1 - 1
● SGOT/AST	23.9 U/L	5 - 34
● SGPT/ALT	30.1 U/L	< 55
SGOT/SGPT Ratio	0.79 %	
● Alkaline Phosphatase	85 U/L	40 - 150
● Total Protein	7.2 g/dL	6.4 - 8.3
● Albumin	4.5 gm/dL	3.8 - 5
● Globulin	2.7 g/dL	2.3 - 3.5
● Albumin :Globulin Ratio	1.67	1 - 2.1
● Gamma Glutamyl Transferase (GGT)	11.2 U/L	12 - 64

KIDNEY PROFILE

Test Name	Result unit	Range
● Blood Urea	34.2 mg/dL	18 - 55
● Bun	15.98 mg/dL	8.4 - 25.7
● Creatinine	0.9 mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	101.47 mL/min/1.73 sq m	
● Bun/Creatinine Ratio	17.76	12 - 20
● Urea / Creatinine Ratio	38	25.68 - 42.8
● Uric Acid	6.7 mg/dL	3.5 - 7.2
● Calcium Serum	9.2 mg/dL	8.4 - 10.2

ELECTROLYTE PROFILE

Test Name	Result unit	Range
● Phosphorus	3.7 mg/dL	2.3 - 4.7
● Sodium	137 mmol/L	136 - 145
● Potassium	4.3 mmol/L	3.5 - 5.1
● Chloride	98.5 mmol/L	98 - 107



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● In Range● Borderline● Out Of Range

No color - Reference range not available

CARDIAC PROFILE

Test Name	Result unit	Range
● Total Cholesterol	192 mg/dL	< 200
● Triglycerides	198 mg/dL	< 150
● HDL Cholesterol	50.7 mg/dL	40 - 80
● Non HDL Cholesterol	141.3 mg/dL	< 130
● LDL Cholesterol	101.7 mg/dL	30 - 100
● V.L.D.L Cholesterol	39.6 mg/dL	< 30
● Chol/HDL Ratio	3.79 Ratio	3.5 - 5
● HDL/ LDL Ratio	0.5 Ratio	0.5 - 3
LDL/HDL Ratio	2.01 Ratio	

IRON

Test Name	Result unit	Range
● Iron	70.1 µg/dL	65 - 175
● TIBC,(Total Iron Binding Capacity)	379.1 µg/dL	250 - 450
● UIBC	309 µg/dL	69 - 240
● Transferrin Saturation	18.49 %	14 - 50

VITAMIN PROFILE

Test Name	Result unit	Range
● Vitamin - B12	404 pg/mL	187 - 883
● Vitamin D 25 - Hydroxy	17.7 ng/mL	30 - 100

THYROID PROFILE

Test Name	Result unit	Range
● Triiodothyronine (T3)	126 ng/dL	35 - 193
● Total Thyroxine (T4)	6.59 µg/dL	4.87 - 11.2
● Thyroid Stimulating Hormone (Ultrasensitive)	2.72 µIU/mL	0.35 - 4.94
● T4, Free	1.01 ng/dL	0.8 - 1.8

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Health Advisory

● In Range ● Borderline (BL) ● Out Of Range



Diabetes

This panel is used to check how much glucose/sugar there is in your blood. Too much blood glucose might indicate diabetes.

Glycosylated Hemoglobin (HbA1c): 5.9 %

● **BORDERLINE**

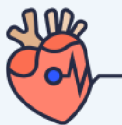
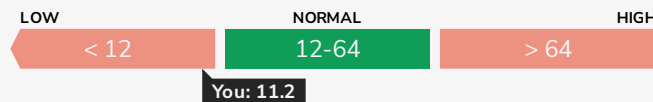


Liver Profile

One of the main functions of your liver is to make proteins that are secreted in your blood. It also makes enzymes which convert food into energy, and processes old muscles and cells. When your liver is damaged, enzymes leak into your blood and appear in the blood test

Gamma Glutamyl Transferase (GGT): 11.2 U/L

● **BORDERLINE**



Cardiac Profile

Most people believe they are safe from heart diseases, but in reality, heart diseases are the leading cause of death in the world. There are many different forms of heart disease. Narrowing or blockage of the coronary arteries is the most common cause of heart disease, which are the vessels that supply blood to the heart. This is called coronary artery disease and it occurs slowly over time. It is the main cause of heart attacks.

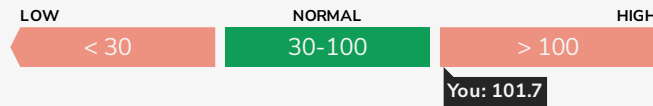
Triglycerides: 198 mg/dL

● **OUT OF RANGE**



LDL Cholesterol: 101.7 mg/dL

● BORDERLINE

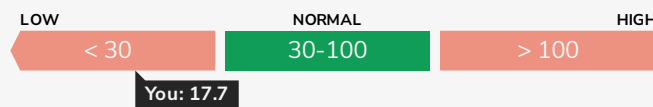


Vitamins Profile

Vitamins are considered essential nutrients because they perform hundreds of roles in your body. They help maintain bones, heal wounds, and strengthen your immune system. They also convert food into energy, and repair cellular damage

Vitamin D 25 - Hydroxy: 17.7 ng/mL

● OUT OF RANGE



Patient NAME : Mr RAJORSHI SEAL

DOB/Age/Gender : 54 Y/Male

Patient ID / UHID : 16370278/OF16370278

Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115876

Sample Type : Whole blood EDTA

Report Date : Apr 16, 2026, 02:26 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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BharatFit Package- 5**Complete Blood Count (CBC)**

RBC Parameters			
Hemoglobin <i>Cyanide Free Spectrophotometry</i>	13.9	g/dL	13.0 - 17.0
RBC Count <i>Electrical impedance</i>	4.4 L*	10 ⁶ /μl	4.5 - 5.5
PCV <i>Electrical impedance</i>	41.2	%	40 - 50
MCV <i>Calculated</i>	94.6	fl	83 - 101
MCH <i>Calculated</i>	32	pg	27 - 32
MCHC <i>Calculated</i>	33.8	g/dL	31.5 - 34.5
RDW (CV) <i>Calculated</i>	13.8	%	11.6 - 14.0
RDW-SD <i>Calculated</i>	51.8 H*	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance</i>	6.8	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils <i>Flow cytometry</i>	64	%	40-80
Lymphocytes <i>Flow cytometry</i>	28.8	%	20-40
Monocytes <i>Flow cytometry</i>	4.9	%	2-10
Eosinophils <i>Flow cytometry</i>	2.1	%	1-6
Basophils <i>Flow cytometry</i>	0.2	%	<2
Absolute Leukocyte Counts <i>Calculated</i>			
Neutrophils.	4.35	10 ³ /μl	2 - 7
Lymphocytes.	1.96	10 ³ /μl	1 - 3
Monocytes.	0.33	10 ³ /μl	0.2 - 1.0
Eosinophils.	0.14	10 ³ /μl	0.02 - 0.5
Basophils.	0.01 L*	10 ³ /μl	0.02 - 0.5
Platelet Parameters			
Platelet Count <i>Electrical impedance</i>	196	10 ³ /μl	150 - 410

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



Dr. Deep Banerjee
WBMC 79005
MBBS, M.D
Consultant Pathologist



Booking Centre :- Peerless Golden Jubilee Charitable Trust, 5/3/A Dumdum Road, Kolkata

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., CG 223, Sector 2, Salt Lake City, 1st & 2nd Floor, Kolkata-700091

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Patient NAME : Mr RAJORSHI SEAL

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Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115876

Sample Type : Whole blood EDTA

Report Date : Apr 16, 2026, 02:26 PM.



Test Description	Value(s)	Unit(s)	Reference Range
Mean Platelet Volume (MPV) <i>Calculated</i>	11	fL	9.3 - 12.1
PCT <i>Calculated</i>	0.2	%	0.17 - 0.32
PDW <i>Calculated</i>	19.8	fL	8.3 - 25.0
P-LCR <i>Calculated</i>	43.2	%	18 - 50
P-LCC <i>Calculated</i>	63	10 ⁹ /L	44 - 140
Mentzer Index <i>Calculated</i>	21.5	%	> 13

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115876

Sample Type : Whole blood EDTA

Report Date : Apr 16, 2026, 03:17 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate MODIFIED WESTERGREN	08	mm/hr	0 - 12
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Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and Lewis practical hematology



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Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115876

Sample Type : Whole blood EDTA

Report Date : Apr 16, 2026, 03:02 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c)	5.9 H*	%	<5.7
Estimated Average Glucose	122.63	mg/dL	Refer Table Below

Interpretation:**Interpretation For HbA1c% As per American Diabetes Association (ADA)**

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Patient ID / UHID : 16370278/OF16370278

Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115857

Sample Type : FLUORIDE F

Report Date : Apr 16, 2026, 02:45 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Blood Sugar Fasting

Glucose Fasting <i>Hexokinase</i>	96.5	mg/dL	70 - 100
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Interpretation:

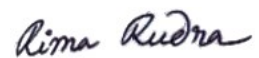
Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥ 126

Reference : American Diabetes Association**Comment :**

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy, insulinoma, or various liver diseases.

Note

1. The diagnosis of Diabetes requires a fasting plasma glucose of ≥ 126 mg/dL or a random / 2 hour plasma glucose value of ≥ 200 mg/dL with symptoms of diabetes mellitus.
2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.



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Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115877

Sample Type : Serum

Report Date : Apr 16, 2026, 02:45 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Liver Function Test (LFT)

Bilirubin Total <i>Diazo</i>	0.331	mg/dL	0.2 - 1.2
Bilirubin Direct <i>Diazo</i>	0.138	mg/dL	0.0 - 0.5
Bilirubin Indirect <i>Calculation (T Bil - D Bil)</i>	0.19	mg/dL	0.1 - 1.0
SGOT/AST <i>IFCC with P5P</i>	23.9	U/L	5 - 34
SGPT/ALT <i>IFCC with P5P</i>	30.1	U/L	0 to 55
SGOT/SGPT Ratio	0.79	-	-
Alkaline Phosphatase <i>p-Nitrophenyl Phosphate</i>	85	U/L	40 - 150
Total Protein <i>Biuret</i>	7.2	g/dL	6.4 - 8.3
Albumin <i>Bromo cresol Green</i>	4.5	gm/dL	3.8 - 5.0
Globulin <i>Calculation (T.P - Albumin)</i>	2.7	g/dL	2.3 - 3.5
Albumin :Globulin Ratio <i>Calculation (Albumin/Globulin)</i>	1.67	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) <i>L-Gamma glutamyl-3-Carboxy-4- Nitroanilide</i>	11.2 L*	U/L	12 - 64

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

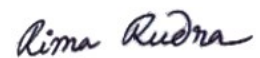
Key enzymes tested:

- 1. AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
- 2. ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
- 3. Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

- 1. Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
- 2. High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
- 3. Low albumin:** Caused by poor diet, kidney, or liver disease.
- 4. High albumin:** Usually due to severe dehydration.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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DOB/Age/Gender : 54 Y/Male

Patient ID / UHID : 16370278/OF16370278

Referred BY : Dr. ANUSREE GANGOPADHYAY

Sample Collected : Apr 16, 2026, 02:01 PM

Report STATUS : Final Report

Barcode NO : RL11115877

Sample Type : Serum

Report Date : Apr 16, 2026, 02:45 PM.



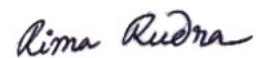
Test Description	Value(s)	Unit(s)	Reference Range
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Kidney Function Test (KFT)

Blood Urea <i>Urease</i>	34.2	mg/dL	18 - 55
Bun <i>Urease</i>	15.98	mg/dL	8.4 - 25.7
Creatinine <i>Creatinase and sarcosine oxidase</i>	0.9	mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	101.47	ml/min/1.73 sq m	Normal Or High: ≥ 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio <i>Calculated</i>	17.76		12 - 20
Urea / Creatinine Ratio <i>Calculated</i>	38		25.68- 42.8
Uric Acid <i>Uricase</i>	6.7	mg/dL	3.5 - 7.2
Calcium Serum <i>NM-BAPTA</i>	9.2	mg/dL	8.4 - 10.2
Phosphorus <i>Phosphomolybdate</i>	3.7	mg/dL	2.3 - 4.7
Sodium <i>Ion selective electrode Indirect</i>	137	mmol/L	136 - 145
Potassium <i>Ion selective electrode Indirect</i>	4.3	mmol/L	3.5 - 5.1
Chloride <i>Ion selective electrode Indirect</i>	98.5	mmol/L	98 - 107

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases. "eGFR test is applicable for patients aged 18 years or more."



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DOB/Age/Gender : 54 Y/Male

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Sample Type : Serum

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Test Description	Value(s)	Unit(s)	Reference Range
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Lipid Profile

Total Cholesterol <i>CHOD-PAP</i>	192	mg/dL	<200
Triglycerides <i>GPO-POD</i>	198 H*	mg/dL	<150
HDL Cholesterol <i>CHER and CHOD</i>	50.7	mg/dL	>40
Non HDL Cholesterol <i>Calculated</i>	141.3 H*	mg/dL	<130
LDL Cholesterol <i>Calculated</i>	101.7 H*	mg/dL	<100
V.L.D.L Cholesterol <i>Calculated</i>	39.6 H*	mg/dL	< 30
Chol/HDL Ratio <i>Calculated</i>	3.79	Ratio	3.5 - 5.0
HDL/ LDL Ratio <i>Calculated</i>	0.5	Ratio	0.5 - 3.0
LDL/HDL Ratio <i>Calculated</i>	2.01	Ratio	-

Interpretation:

Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year,. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.

Causes of variation include

- Diet: Levels spike 5–10 times after meals. Unhealthy high saturated fat diets like non veg diet sweetened beverages , fried or processed foods , and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity , sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like : Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol
Low
<40
High
>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Test Description		Value(s)	Unit(s)	Reference Range
Risk Category	A. CAD with > 1 feature of high risk group			
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease			
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia			
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque			
Moderate Risk	2 major ASCVD risk factors			
Low Risk	0-1 major ASCVD risk factors			
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors				
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use			
2. Family history of premature ASCVD	4. High blood pressure			
5. Low HDL				

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.
- Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637-1646.
- P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van & Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis.
- Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825-32. DOI: 10.3238/arztebl.2019.0825

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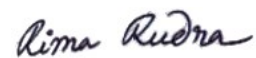
Iron Studies

Iron <i>FerroZine</i>	70.1	µg/dL	65 - 175
TIBC,(Total Iron Binding Capacity) <i>Calculated</i>	379.1	µg/dL	250 - 450
UIBC <i>Ferrozine</i>	309 H*	µg/dL	69 - 240
Transferrin Saturation <i>Method :Derived from IRON and TIBC values</i>	18.49	%	-

Interpretation:

Increased levels due to iron ingestion or ineffective erythropoiesis. Decreased levels due to infection, inflammation, malignancy, menstruation and Fe deficiency. Needs to be taken into consideration with TIBC. Transferrin Saturation:- Low level Transferrin Saturation can indicate iron deficiency, erythropoiesis, infection, or inflammation. High level Transferrin Saturation can indicate recent ingestion of dietary iron, ineffective erythropoiesis, haemochromatosis or liver disease. High TIBC, UIBC, or transferrin usually indicates iron deficiency, but they are also increased in pregnancy and with the use of oral contraceptives. Low TIBC, UIBC, or transferrin may occur if someone has: Hemochromatosis, Certain types of anemia due to accumulated iron, Malnutrition, kidney disease that causes a loss of protein in urine.

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Test Description	Value(s)	Unit(s)	Reference Range
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Vitamin B12 / Cyanocobalamin

Vitamin - B12 ECLIA	404	pg/mL	187 - 883
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Interpretation:

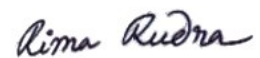
Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.

Causes of vitamin B12 deficiency include: Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."



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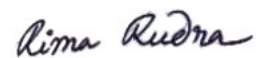
Vitamin D 25 Hydroxy

Vitamin D 25 - Hydroxy ECLIA	17.7 L*	ng/mL	Deficiency : <30 ng/mL
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Interpretation:

25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)



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Thyroid Profile Total

Triiodothyronine (T3) ECLIA	126	ng/dL	35 - 193
Total Thyroxine (T4) ECLIA	6.59	µg/dL	4.87 - 11.2
Thyroid Stimulating Hormone (Ultrasensitive) ECLIA	2.72	µIU/mL	0.35 - 4.94

Interpretation:

Pregnancy	Reference Range TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Note: TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance

Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.

2. Secondary/Tertiary Hypothyroidism: Low TSH levels.

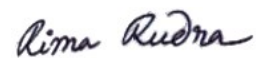
3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Normal	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

References

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568



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FT4 & TSH 3rd Gen**FT4 (Free Thyroxine 4)**

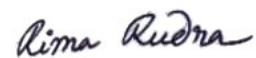
T4, Free ECLIA	1.01	ng/dL	0.8 - 1.8
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Interpretation:

1. Free T4 is the unbound and active form of thyroxine, a hormone produced by the thyroid gland that helps regulate metabolism, energy production, and overall growth and development. Interpreting free T4 levels helps diagnose thyroid function disorders.

2. Low free T4 levels, indicative of hypothyroidism, can result from several causes. Primary hypothyroidism occurs when the thyroid gland itself is underactive, often due to Hashimoto's thyroiditis, iodine deficiency, or thyroid surgery. Secondary hypothyroidism arises when the pituitary gland fails to produce sufficient thyroid-stimulating hormone (TSH) to activate the thyroid, commonly due to pituitary disorders. Tertiary hypothyroidism is caused by the hypothalamus failing to produce enough thyrotropin-releasing hormone (TRH), which leads to decreased production of TSH and subsequently T4.

3. High free T4 levels, indicative of hyperthyroidism, can be caused by various conditions. Graves' disease, an autoimmune disorder, overstimulates the thyroid gland, leading to excessive hormone production. Overactive thyroid nodules can also contribute to high free T4 levels by producing excess hormone independently. Thyroiditis, an inflammation of the thyroid, can release stored hormones into the bloodstream, causing elevated levels. Overmedication with thyroid hormone replacement can also result in high free T4 levels.



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TSH, Ultrasensitive

Thyroid Stimulating Hormone (Ultrasensitive) ECLIA	2.72	μIU/mL	0.35 - 4.94
---	------	--------	-------------

Interpretation:

Pregnancy	Reference ranges TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Note:

TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:

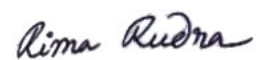
- Diagnose Hypothyroidism and Hyperthyroidism
- Monitor T4 replacement or T4 suppressive therapy
- Quantify TSH levels in the subnormal range

Increased Levels : Primary hypothyroidism, Subclinical hypothyroidis, TSH dependent Hyperthyroidism, Thyroid hormone resistance

Decreased Levels: Grace disease, Autonomous thyroid hormone secretion, TSH deficiency

References:

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568



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BharatFit Package- 5**Hepatitis B Surface Antigen (HBsAg) Rapid Screening Test**

HEPATITIS B SURFACE ANTIGEN (HBsAg) <i>Qualitative immunoassay, rapid card</i>	NON REACTIVE		NON REACTIVE
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Interpretation:

RESULTS	REMARKS
Reactive	The sample is Reactive for HBsAg
Non Reactive	The sample is Non Reactive for HBsAg

Note

1. **This is only a Screening test.** All reactive results should be confirmed by confirmatory test. Therefore for a definitive diagnosis, the patient's clinical history, symptomatology as well as serological data, should be considered. The results should be reported only after complying with above procedure.
2. Additional follow up testing using available clinical methods (along with repeat HBsAg rapid card test) is required, if the test is Non reactive with persisting clinical symptoms
3. False positive results may be observed in patients receiving mouse monoclonal antibodies, on heparin therapy, on biotin supplements for diagnosis or therapy, presence of heterophilic antibodies in serum or after HBV vaccination for transient period of time.
4. False negative reaction may be due to processing of sample collected early in the course of disease or presence of mutant forms of HBsAg.

*** End Of Report ***

Pending Report To Follow - Urine R/M

Paulami Ghosh
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MD. Microbiology
Consultant Microbiologist

Booking Centre :- Peerless Golden Jubilee Charitable Trust, 5/3/A Dumdum Road, Kolkata

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., CG 223, Sector 2, Salt Lake City, 1st & 2nd Floor, Kolkata-700091

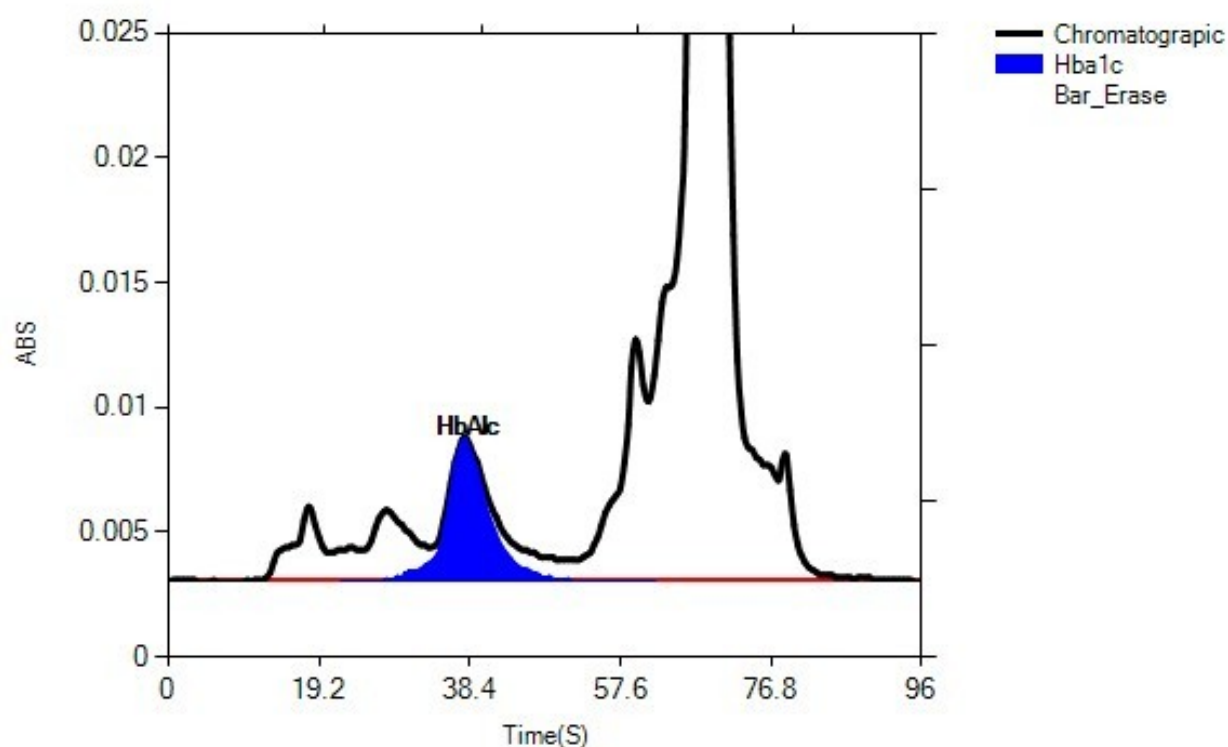
 928-909-0609 ccsupport@redcliffelabs.com www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

ADPL HbA1c Graph Report

Name :	Sample Id :	RL11115876
Sample Type :	Whole Blood EDTA	Total Area : 7.974

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
HbA0	68	0.1833	7.191	91
HbA1c	38	0.005	0.519	5.9
La1c	36	0.0055	0.095	1.1
HbF	22	0.0013	0.074	0.9
Hba1b	16	0.0015	0.082	1.0
Hba1a	12	0.0002	0.013	0.1



Terms and Conditions of Reporting

1. The presented findings in the Reports are intended solely for informational and interpretational purposes by the referring physician or other qualified medical professionals possessing a comprehensive understanding of reporting units, reference ranges, and technological limitations. The laboratory shall not be held liable for any interpretation or misinterpretation of the results, nor for any consequential or incidental damages arising from such interpretation.
2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.
3. It is to be noted that variations in results may occur between different laboratories and over time, even for the same parameter for the same Customer. The assays are performed and conducted in accordance with standard procedures, and the reported outcomes are contingent on the specific individual assay methods and equipment(s) used, as well as the quality of the received specimen.
4. This report shall not be deemed valid or admissible for any medico-legal purposes.
5. The Customers assume full responsibility for apprising the Company of any factors that may impact the test finding. These factors, among others, includes dietary intake, alcohol, or medication / drug(s) consumption, or fasting. This list of factors is only representative and not exhaustive.

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TEST PARAMETERS

- | | |
|-----------------------------------|--------------------------|
| ✓ Blood Sugar Fasting (1 Test) | ✓ ESR (1 Test) |
| ✓ Lipid Profile (9 Tests) | ✓ HbA1c (2 Tests) |
| ✓ Liver Function Test (12 Tests) | ✓ Vitamin D (1 Test) |
| ✓ Kidney Function Test (12 Tests) | ✓ Vitamin B12 (1 Test) |
| ✓ Thyroid Profile Total (3 Tests) | ✓ Iron Studies (4 Tests) |
| ✓ Urine R/M (23 Tests) | ✓ HBsAg (Rapid) (1 Test) |
| ✓ Complete Blood Count (26 Tests) | |



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